

SUBJECTIVE:

Patient is a 20-year-old presenting with chief complaint of horrendous constipation for the past week. She reports her usual bowel movement frequency is daily, but she has not had a bowel movement for the past seven days. She describes extreme pain localized in her colon. She experiences significant straining during attempted bowel movements. The patient reports associated rectal bleeding, described as bright red, occurring both during and after these attempts. She notes that any stool attempted to be passed, though minimal, feels soft.

OBJECTIVE:

Expected Physical Exam Findings:

Abdominal examination may reveal distention, diffuse tenderness to palpation, and potentially hyperactive or diminished bowel sounds. Tympany on percussion is possible.

Rectal examination may show external hemorrhoids, anal fissures, or excoriations. Digital rectal exam is likely to reveal a firm, palpable fecal mass in the rectal vault. Bright red blood may be noted on the examining glove.

Vital Signs: Unavailable.

ASSESSMENT:

Primary Diagnosis:

Severe Constipation, likely with Fecal Impaction.

Differential Diagnoses:

- Anal Fissure
- Hemorrhoids
- Obstructive Colorectal Mass (consider given new bleeding and acute severe change in bowel habits)
- Irritable Bowel Syndrome with Constipation (IBS-C)
- Medication-induced constipation

PLAN:

Tests:

- Abdominal X-ray (KUB) to assess for fecal impaction and rule out obstruction.
- Complete Blood Count (CBC) to evaluate for anemia and infection.
- Basic Metabolic Panel (BMP) to check electrolytes.
- Stool occult blood test.
- Consider referral for flexible sigmoidoscopy or colonoscopy after acute resolution to evaluate bleeding source and rule out structural pathology.

Medications:

- Osmotic laxatives (e.g., Polyethylene Glycol).
- Stool softeners (e.g., Docusate Sodium).
- Rectal suppositories or enemas for disimpaction (e.g., Bisacodyl suppository, Fleet enema).
- Analgesia as needed for pain.

Patient Advice:

- Increase oral fluid intake.
- Gradually increase dietary fiber once bowel movements resume.
- Avoid excessive straining.
- Encourage light activity if tolerated.

Follow-up instructions:

- Return to clinic or emergency department promptly for worsening abdominal pain, intractable vomiting, fever, or increased rectal bleeding.
- Follow-up with provider within 24-48 hours to assess treatment effectiveness.
- Arrange for a colonoscopy consultation once acute symptoms resolve.